Ankle & Foot Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: legs exposed to the knees; shoes and socks off; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both limbs, front, side and back			
☐	Gait	Normal? Antalgic?	
☐	Swelling, scars, deformity	Ankle and foot	
☐	Wasting	Calf, small foot muscles	
☐	Nails	Psoriatic changes	Psoriatic arthritis
☐	Arches	Transverse (under MTPs) and longitudinal (1st MTP to heel)	Flattened in RA (pes planus)
☐	Calluses	Soles	Abnormal pressure
☐	Forefoot	Hallux valgus, clawing, crowding of toes, sausage toes	Bunion; RA; dactylitis
☐	Footwear*	Wear pattern on shoes †	
Feel			
☐	Temperature	Compare both sides	Warm = inflammation, gout, infection
☐	Ankle joint line	Swelling, tenderness	
☐	Malleoli and ligaments	Both malleoli; medial and lateral ligaments	Fracture, sprain
☐	Base of 5th metatarsal	Palpate	Avulsion fracture
☐	Achilles tendon	Nodules, tenderness, gap †	Tendinitis, rupture, RA nodules
☐	MTP squeeze	Compress 1st and 5th metatarsals together between thumb and finger	Tender = MTP synovitis (early RA)
☐	MTP and IP joints	Palpate individually	Tenderness, swelling; 1st MTP = gout †
Move			
☐	Ankle	Dorsiflexion and plantarflexion (active, then passive)	
☐	Subtalar joint	Inversion and eversion (hold the heel †)	
☐	MTP and IP joints	Passive flexion and extension of each	Hallux rigidus †
Special tests			
☐	Knee-to-wall	Heel flat, bend knee to touch wall; move foot back until heel lifts or pain; measure great toe to wall	Normal ≥ 9 cm; less = reduced dorsiflexion
☐	Thompson / Simmonds	Patient kneels on a chair; palpate Achilles; squeeze calf just below its widest point	No plantarflexion = Achilles rupture
☐	Mulder's sign	Squeeze the metatarsals together and press up from the sole between 3rd and 4th toes, just proximal to MTPs	Pain (± click) = Morton's neuroma

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the knee, do a neurovascular exam of the foot, and assess footwear †."
☐	Present: "Normal gait. No swelling, deformity or skin changes; arches preserved. No tenderness, including over the malleoli and base of the 5th metatarsal. Full range at the ankle, subtalar and toe joints. Knee-to-wall over 9 cm; Thompson and Mulder's tests negative."

4. Quick reference †

- **Ottawa ankle rules**: X-ray if malleolar pain plus bone tenderness at the posterior edge or tip of either malleolus, or unable to weight-bear 4 steps. Foot X-ray if tender at the base of the 5th metatarsal or navicular.
- Acute hot, swollen 1st MTP = think gout; exclude septic arthritis if febrile or unwell.